

PROBLEMS: Solved in Real Time

1. Problem occurs
(non-ideal care)
2. Process observed
and studied
3. Participants
interviewed
4. All by front line staff
and mgr
5. A3 Problem Solving

The Power of OBSERVATION

- Observe Process
- Observe the Data
- Observe Timing
- Observe Defects
- Observe Wastes
- Interview the observed
- By the frontline staff

Value Stream Mapping

- Graphic map of:
 - Steps in request
 - Steps in Work Flow
 - (with variation measured & displayed)*
 - Pathway to Satisfy the Request
- Current State Map
- Future State Map
- Future State Plan

Value Stream Maps (VSM)

- Customer with a request is the only starting point—and only ending point
- Requests are mapped as a process
- Delays and variation are measured
- Frustrations, barriers, & waste = “storm clouds”
- Shared & validated publically = buy in up front
- *It seems the whole system map is emerging from the problem solving of the front line.*

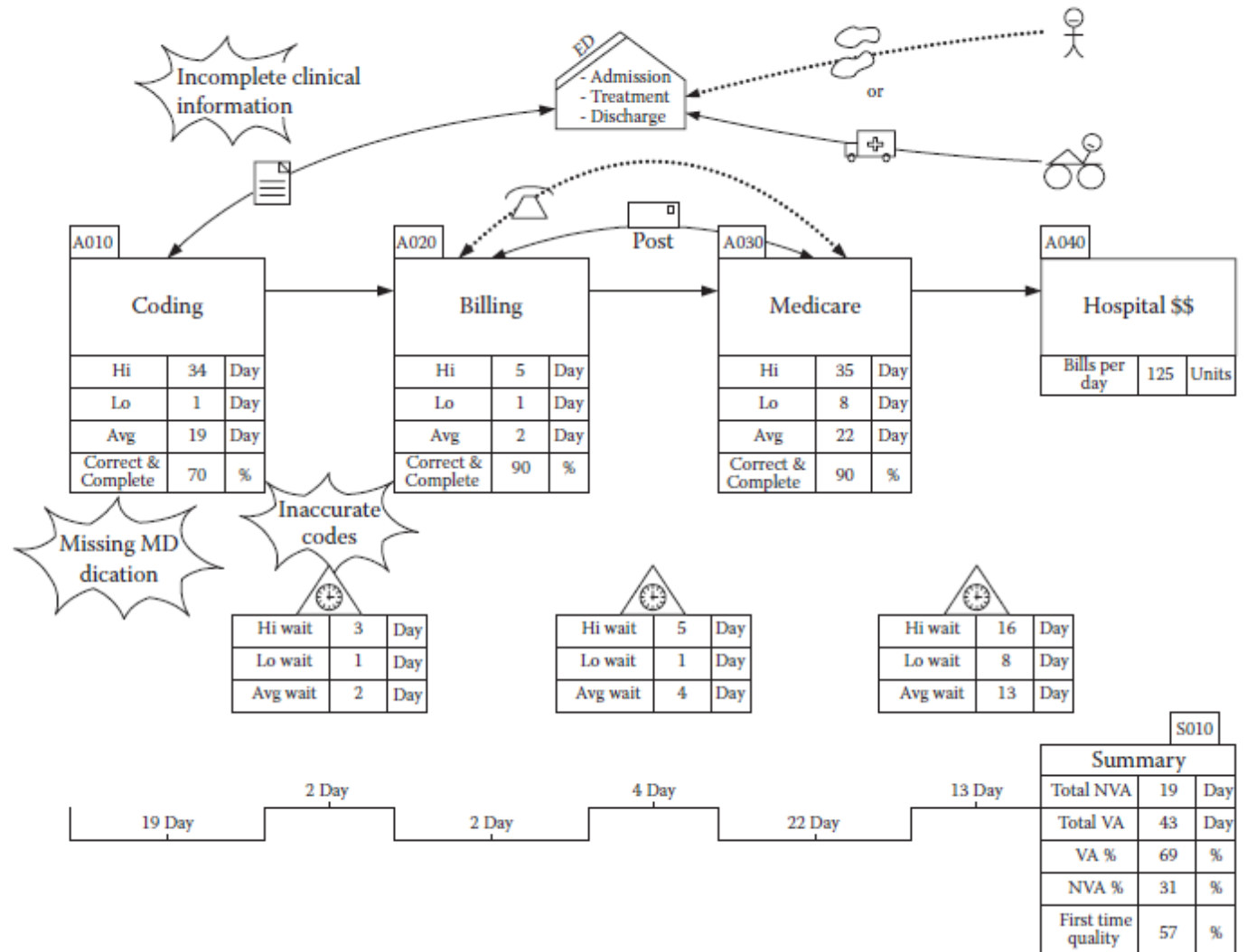
VSM

High Level

-Customer
-Request
-Value Add
-Non Value
-Delays
-Customer
satisfaction

-Targeted
problems

Title : Medicare Billing for Emergency Dept. Charges
Date : May 03, 2003



A3 Problem Solving

- ❑ Hand out blank A3 forms & provide MS Word documents
- ❑ Share examples

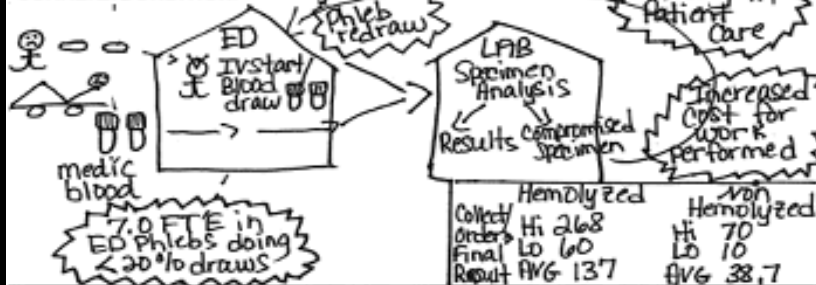
ISSUE:	TARGET CONDITION: TITLE: TO: BY: DATE:
BACKGROUND:	
CURRENT CONDITION:	
PROBLEM ANALYSIS:	COUNTER MEASURES:
	IMPLEMENTATION PLAN: WHAT WHO WHEN OUTCOME:
	COST: COST BENEFIT/WASTE RECOGNITION:
	TEST:
	FOLLOW UP:

A3 Lab Blood Draws in ED

ISSUE: Patient care is delayed when compromised lab specimens are sent to the lab for analysis.

BACKGROUND: Decreased hemolysis and blood culture contamination rates are seen when blood is drawn by a phlebotomist rather than a nurse or medic.

CURRENT CONDITION:



PROBLEM ANALYSIS:

Delay in Patient Care

why? Compromised specimens drawn by nurse/medics

why? Drawn by IV Start

why?

Phlebs stationed in ED drawing < 20% Specimens

why? Nurses/medics drawing blood

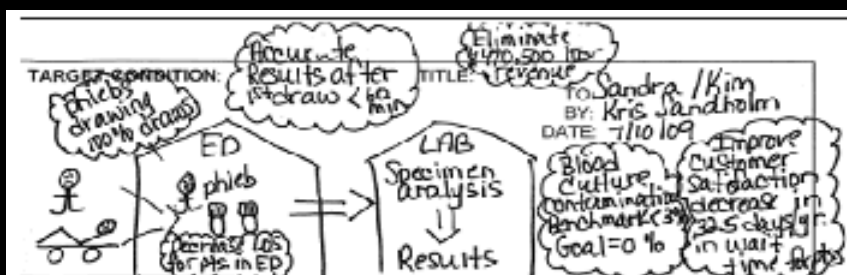
Increased Cost for work performed

why? Compromised specimens require rework

why? Need to be redrawn or worked up

why? Compromised specimens sent for analysis

why? Blood drawn through IV starts



COUNTER MEASURES:

- Optimize use of current phlebotomy staff in ED.
- Communicate at ED staff meeting - cessation of nurse blood draws including blood cultures.
- Communicate initiation of 100% of ED draws by phlebotomists.

IMPLEMENTATION PLAN:

WHAT:	WHO:	WHEN:	OUTCOME:
Cessation of blood draws by nurses/medics	Laura	7/18/09 0700	phlebs drawing 100% draws

Initiate 100% phlebs draws in ED	Elton/Kim	7/18/09 0700	✓
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Relocate In-house phlebotomy home base to ED	Susan/Laura	8/1/09	Optimize use of phlebotomy team
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COST:	COST BENEFIT/WASTE RECOGNITION:	year
	Elimination of overdraw	\$1,500
	Blood Culture Contamination	\$35,000
	Rejected Specimens	\$2000
	Total	\$38,500

TEST: Implementation 7/18 with scheduled follow ups.

FOLLOW UP:

- Elton/Laura communicate for first few days following
- Starting reporting forms for shortages.
- Team follow up 1 week, 3 weeks and quarterly for 1st year.

QUICK REFERENCE A3 TEMPLATE

ISSUE: what is the issue through the eyes of the customer/patient?

BACKGROUND

- information for understanding the problem
- importance of the problem

CURRENT CONDITION

- diagram of how the work happens now
- highlight problems with storm clouds
- what about the problem is NOT defect-free?
- Can you measure the waste?

CAUSE ANALYSIS

- List problems identified in box above (storm clouds)
- Ask WHY? 5 times to get to most likely direct cause

TARGET CONDITION

- Diagram of a better way to do the work
- Good features highlighted as fluffy clouds
- measurable targets (quantity/time)

TITLE:

TO:

BY:

DATE:

COUNTERMEASURE(S)

- WHAT are we going to do to get to the Target Condition

IMPLEMENTATION PLAN

- (HOW are we going to get there?)
_Action _Who _When _Outcome

Cost:

Cost benefit/waste reduction:

TEST: can you design an experiment to test your implementation plan?

FOLLOW-UP:

- use a red box for easy visibility
- date the follow-up was done
- actual results compared to anticipated results
(this becomes the NEW current condition)